

NHS Procurement Route Response Pack

A source-led copy and positioning pack built from the HCI / P4H NHS supplier guidance, designed to help OTOS answer the procurement route before the buyer asks the question.

Core procurement answer

OTOS is not asking the NHS to buy a finished clinical system.

OTOS is preparing a low-disruption, non-clinical continuity demonstrator that aligns with prevention, lower acuity, administration reduction, measurable social value and buyer-safe collaborative procurement routes.

Draft status: v0.1 - working copy blocks and route hypothesis. Not legal advice. Not a tender response. Not final external copy.

Primary new input: HCI / P4H "Winning Work within the NHS: A Supplier's Guide to Collaborative Procurement" webinar slides.

Current document context: OTOS review wall / repo contains 13 current HTML document files covering NIA Paper, Angel Brief, Executive Overview, AZ Master Brief, Evidence Dossier, ADHD Team, CGL/CRS, Alcohol Liaison, Dr Claire/LAMP, RCE, Mind/WorkWell, Public Health and Champion Briefs.

1. Executive read: why this matters

The HCI/P4H material reframes the NHS route from “how do we persuade a service?” to “how do we position OTOS as a procurement-ready, intelligence-led, low-burden supplier before the tender window opens?”

The important shift is not just procurement. It is route discipline.

Direct route hypothesis

OTOS should create a procurement-facing layer of its document pack that answers: **why this fits NHS priorities, why it suits collaborative procurement, why it is low-risk for buyers, why it has measurable social value, and why early engagement is justified before any ITT.**

The HCI deck says buyers respond to insight-led, specific, priority-linked outreach, not generic “give me half an hour” messages. It also presents 2026 as a major re-competition and framework/market-access window. For OTOS, that creates a practical route: package the same truth spine into a procurement-facing response pack, then use HCI/PIN/framework intelligence to identify the correct buyer conversations.

HCI/P4H signal	What OTOS should answer
NHS buyers want relevance to NHS priorities and the 10-Year Plan.	Show the OTOS fit to prevention, lower acuity, reduced admin, standardisation, flow and better follow-through.
Regional hubs suit complex clinical software, wraparound propositions and implementation assurance.	Position OTOS as a complex, non-clinical continuity layer requiring governance, implementation confidence and partner-safe onboarding.
PINs and early market engagement matter.	Prepare an early-market-engagement pack before OTOS becomes locked out by framework timing.
Social value is scored and must be specific, measurable, accountable and transparent.	Turn founder-led lived experience, local workforce, VCSE alignment and neurodivergent access into SMART commitments.
SMEs and VCSE suppliers are being actively discussed.	Present OTOS as a credible SME innovation partner with a bounded demonstrator rather than an unstructured start-up pitch.

2. What the HCI material says - translated into OTOS language

These are not final external claims. They are translation points to use when strengthening the existing OTOS PDFs.

Theme	HCI/P4H meaning	OTOS answer
Scale / timing	The deck frames the NHS supplier market as unusually open, with large expiring pipelines, frameworks and a re-competition window.	OTOS should not wait until “clinical interest” is perfect. It needs a procurement-intelligence route running in parallel.
Buyer relevance	The deck says generic outreach is ignored; what works is knowledge of buyer markets, programmes and priorities.	Each OTOS document should include a one-paragraph “why this buyer, why now, why this route” block.
10-Year Plan fit	The deck lists priorities: cut waiting times, automate manual tasks, prevent illness, standardise/rationalise and reduce cost.	OTOS should explicitly answer these using continuity visibility, drift reduction, reduced admin chasing, lower-acuity support and structured handoffs.
Hub fit	Hubs sit between local and national buying and suit complex, wraparound, implementation-heavy propositions.	OTOS should be framed as a complex pathway / continuity implementation, not a simple app or content product.
Pre-tender approach	The deck emphasises early PINs, spend analysis, incumbents and informed outreach before tenders land.	OTOS needs a named target-list and early-market-engagement version of the brief.

Theme	HCI/P4H meaning	OTOS answer
Social value	The deck says credible social value is specific, measurable, accountable and transparent.	OTOS needs a scored social-value page, not a vague mission statement.

3. The procurement-facing OTOS answer

Recommended headline

OTOS Continuity™: a low-disruption, non-clinical continuity layer for earlier visibility, safer handoffs and measurable pathway learning around existing services.

This line is procurement-safe because it avoids clinical outcome claims while still answering the buyer's practical question: what is it, why does it matter, and what does it do?

Short procurement-positioning paragraph

OTOS Continuity™ is a non-clinical continuity layer designed to sit around existing NHS, addiction, mental-health, VCSE and community services. It does not diagnose, prescribe, treat, replace clinical decision-making or write into NHS records. Its first value is process visibility: handoff receipts, drift signals, consent-led engagement prompts, human review and an auditable learning trail that helps partners see whether the next step happened after a referral, signpost, discharge or wait-list point.

Use note: Use in Executive Overview, Public Health, ADHD Team, CGL/CRS and procurement-facing cover notes.

Buyer-safe one-liner

We do not deliver treatment. We run the pathway layer that helps existing treatment, support and community touchpoints stay connected.

Use note: Use wherever "what is OTOS?" needs to be answered fast.

Procurement route one-liner

OTOS is not asking for a full system purchase; it is asking for a bounded, low-burden demonstrator that can produce evidence before procurement, scale or clinical claims.

Use note: Use in emails to buyers, HCI/P4H follow-up, Health Innovation and local authority conversations.

4. Answers to NHS 10-Year Plan / transformation priorities

Use these as copy blocks across the existing PDF family. They almost answer the buyer's implied question: "which NHS priority does this help?"

NHS priority	OTOS answer
Cut waiting times	OTOS does not shorten clinical waiting lists directly. It supports the waiting period by keeping the person connected, prompting structured engagement and making drift visible before the next crisis contact.
Prevent illness / lower acuity	OTOS is positioned upstream of repeated crisis: it aims to test whether earlier continuity around a suspected ADHD-driven addiction cohort can reduce avoidable escalation.
Automate manual tasks	OTOS can reduce the hidden admin of chasing whether a handoff landed, whether a person engaged, or whether silence followed discharge - while keeping human review in control.
Standardise and rationalise	OTOS creates a repeatable continuity process around services that currently improvise around the person. The service stays the service; the handoff becomes visible and auditable.
Patient flow / discharge follow-through	The bedside, discharge and post-signpost moment becomes a receipted transition, not an unsupported instruction to "go and wait."
Value-based / whole-life-cycle commercial model	The demonstrator is not a feature sale. It is a process proof: onboarding, handoff, engagement, drift review, partner learning and evaluation pack.

10-Year Plan alignment paragraph

OTOS aligns with NHS transformation priorities by focusing on prevention, lower acuity, reduced administrative drag and safer continuity around existing services. It does not ask the NHS to adopt a new clinical pathway before evidence exists. It asks to test whether a light, consent-led continuity layer can make waiting, handoff and post-discharge drift visible enough to inform better decisions.

Use note: Use in procurement-facing Executive Overview and Public Health Cost Case.

5. Why OTOS fits collaborative procurement better than a cold local sale

The HCI material is especially useful because it explains why regional hubs and frameworks matter for complex offers. OTOS is not a commodity. It is a pathway implementation with governance, partner coordination, human review and evidence design.

Complexity-fit paragraph

OTOS is a complex, wraparound proposition rather than a simple digital catalogue product. Its value depends on careful implementation: consent, partner language, data boundaries, human review, governance, handoff receipts and local pathway learning. That makes it better suited to early collaborative engagement, hub intelligence and a demonstrator route than to a generic cold procurement approach.

Use note: Use in procurement route pack, Business Plan, Public Health and investor materials.

Implementation assurance paragraph

The demonstrator is designed to de-risk implementation before scale. The first question is not whether OTOS can become a national system. The first question is whether a small, governed, 50-person pathway visibility test can operate with low partner burden and produce useful evidence.

Use note: Use in Angel / funder / NHS conversations.

No disruption paragraph

OTOS is deliberately designed not to disrupt existing services. It sits around them. No merged EPR. No shared clinical notes. No automated clinical decision-making. Human decisions only. The demonstrator tests whether continuity can be made visible without asking partners to change their core clinical responsibilities.

Use note: Use in all partner PDF boundary pages.

6. Social value answer - make it scored, not sentimental

The HCI slides are clear: social value must be specific, measurable, accountable and transparent. OTOS already has a strong social mission, but procurement needs it expressed as commitments.

Scored element	OTOS procurement-ready answer
Specific	Focus on the high-need ADHD + addiction / neurodivergent recovery cohort visible across addiction, mental health, VCSE, community and hospital touchpoints.
Measurable	Report engagement, handoff receipt completion, missed-response patterns, partner burden, participant retention signals and final learning themes.
Accountable	Name the OTOS delivery owner, partner review points, safeguarding escalation boundaries and evaluation lead before launch.
Transparent	Produce a weekly one-page dashboard and a final claims-safe evaluation pack: what worked, what did not, continue/pivot/stop.
Local value	Use lived-experience design, local VCSE/community pathways and local workforce development around neurodivergent-accessible support.

Social value paragraph

OTOS delivers social value by turning lived experience into a measurable continuity process for a high-need, high-friction cohort. The demonstrator can report specific outputs: who was onboarded, which handoffs were receipted, where silence appeared, what partner burden was created, what support touchpoints were used, and what learning should continue, pivot or stop.

Use note: Use in procurement packs and funding applications.

Jobs and skills paragraph

A demonstrator can include a jobs-and-skills commitment by training a small local delivery group in neurodivergent-accessible engagement, consent-led pathway support, safeguarding boundaries, data minimisation and evidence capture. This creates capability around the pathway rather than only buying a technology product.

Use note: Use where jobs/skills weighting or SME/VCSE access matters; verify eligibility before use in tenders.

7. Outreach route - direct but not generic

Procurement route principle

Do not send “give me half an hour.”

Send a buyer-aware message that names the relevant NHS priority, the procurement route question, the demonstrator boundary and the specific ask for routing advice.

Buyer opening email block

I am writing because OTOS Continuity appears to sit in the gap between three NHS priorities that are now difficult to separate: prevention, lower acuity and reducing the administrative drag created when people are handed between services but not held between them. We are not asking for procurement or adoption at this stage. We are asking for the right route to test a bounded, non-clinical, low-disruption demonstrator that can evidence whether handoff receipts, drift visibility and human review can work safely around existing services.

Use note: Use as a building block in HCI/P4H/LPP/Health Innovation outreach.

Specific ask block

Could you advise which route is most appropriate for a pre-procurement conversation: early market engagement, a relevant framework/PIN route, innovation pathway, regional hub discussion, local authority prevention route or partner-led demonstrator? We are specifically trying to avoid generic outreach and reach the correct buyer or innovation lead before any tender conversation is appropriate.

Use note: Use in emails; keeps tone buyer-aware.

P4H / HCI follow-up block

The HCI/P4H material suggests the window for informed, early supplier engagement is unusually important in 2026. OTOS is preparing its evidence and governance pack so that, if the fit is real, the route is clear before the market window closes.

Use note: Use only where referencing HCI/P4H material is appropriate.

8. Direct route workplan

This is the practical route that falls out of the HCI deck.

Step	Action	Output
1	Build a procurement-facing Executive Overview	One page: priority fit, low-disruption demonstrator, what OTOS is/is not, social value, ask.
2	Create buyer-specific versions	NHS hub / LPP, ICB, local authority, Health Innovation, VCSE-led partner, pharma/strategic.
3	Track PINs and frameworks	Use HCI / Find a Tender / hub intelligence to identify prevention, digital health, pathway, mental health, addiction, population health and engagement routes.

Step	Action	Output
4	Map incumbent and route	Before outreach, identify existing contracts, buyer priorities, incumbent suppliers and framework channels.
5	Prepare social value appendix	Specific, measurable, accountable, transparent commitments; not broad mission language.
6	Send route-advice outreach	Ask for routing/market-engagement advice, not procurement or endorsement.
7	Use P4H as a buyer-intelligence event	Attend/monitor to meet buyers, discuss PINs, challenge SME barriers and refine the framework route.

9. Blocks to inject into the existing PDF family

Use this page as the lift-and-place bank. Do not insert all blocks everywhere. Choose only the lines that match each document's audience.

Document	Add / strengthen
Executive Overview	Add the procurement answer: "not asking for adoption; asking to test a low-disruption continuity demonstrator."
Public Health Cost Case	Add "the system is already paying, but paying late" plus prevention/lower-acuity / social value / local evidence alignment.
Angel / Funder Brief	Add "procurement route is opening" and "evidence before procurement" as commercial-risk reduction.
ADHD Team Brief	Add "clinical decisions remain with qualified teams; OTOS only evidences handoff/drift around the pathway."
CGL / CRS Brief	Add "No Wrong Door becomes No Lost Handoff" and procurement-fit language for VCSE/SME social value.
Alcohol Liaison	Add "discharge follow-through becomes a receipted transition, not a silent handoff."
RCE / Wellbeing Hub	Add "course is not just content; it becomes a measurable touchpoint during waiting."
Mind / WorkWell / Therapy	Add "VCSE contribution can be visible without sharing therapy content or clinical notes."
AZ / Roche	Add procurement intelligence as route logic: early market engagement, evidence pack, data, pathway visibility.
Evidence Dossier	Add a procurement evidence tab: HCI/P4H source notes, social value evidence, framework/PIN route, NHS priority map.

10. Six ready-to-use answer blocks

A. Why now?

The NHS is being asked to deliver prevention, lower acuity, shorter waits, better flow and lower administrative burden while procurement routes are becoming more transparent and more open to innovation, SMEs and social value. OTOS sits precisely where those pressures meet: the gap between services, where people are known but not held.

Use note: Use in Executive Overview / Business Plan / procurement pack.

B. What is the procurement fit?

OTOS is not a commodity item and should not be treated like one. It is a pathway continuity implementation with governance, consent, human review, partner language and evidence design. That makes early collaborative procurement advice essential before any tender route is chosen.

Use note: Use in LPP / hub / framework route conversations.

C. What is the risk control?

The demonstrator controls risk by keeping OTOS outside diagnosis, prescribing, treatment, clinical decisions and NHS record-writing. It tests only whether a continuity layer can create handoff receipts, drift visibility and useful learning around existing services.

Use note: Use in governance pages.

D. What is the buyer value?

The buyer value is practical: less invisible drift, clearer handoff evidence, fewer unsupported transitions, lower administrative uncertainty and a claims-safe evidence pack that shows whether the pathway is worth continuing, pivoting or stopping.

Use note: Use in Public Health / commissioner copy.

E. What is the social value?

The social value is specific: a founder-led, lived-experience-informed demonstrator for a high-need neurodivergent recovery cohort, with measurable outputs, named accountability, transparent reporting and local pathway learning.

Use note: Use in tender/social value/funder pages.

F. What is the first ask?

The first ask is not procurement. It is routing: who is the correct buyer, innovation lead, hub contact, framework owner or early-market-engagement route to pressure-test a safe demonstrator before any purchase decision exists?

Use note: Use in email outreach.

11. Draft mini-brief: “Why OTOS, why this route?”

The following is a compact mini-brief that can be mined directly into new PDFs.

Mini-brief

OTOS Continuity™ answers a procurement problem hidden inside a human pathway problem.

The NHS and its partners are not short of services; they are short of continuity between them. People can be referred, signposted, discharged, supported and placed on waiting lists, yet still disappear in the silent space after the handoff. For the ADHD-driven addiction cohort, that silence can be where waiting becomes drift and drift becomes crisis.

OTOS is designed as a low-disruption, non-clinical continuity layer. It does not diagnose, prescribe, treat, automate clinical decisions or write into NHS records. It creates handoff receipts, drift visibility, consent-led engagement prompts, human review and a learning trail around existing services.

The procurement logic is clear: this is not a finished national system asking to be bought. It is a bounded demonstrator asking to be routed properly. The first test is whether 50 people across 12 weeks can be supported through a safe continuity process that produces useful evidence without burdening partners or changing clinical responsibility.

In the language of NHS procurement, OTOS aligns with prevention, lower acuity, administrative efficiency, standardisation, measurable social value and early supplier engagement. The correct next step is not a generic pitch. It is a route-aware conversation with the right procurement, innovation or hub lead before the tender window arrives.

12. Claims and source-control notes

Keep these notes attached to any future PDF revision using this material.

- Do not describe this as legal procurement advice.
- Do not say the NHS procurement route is guaranteed.
- Do not say OTOS is eligible for a specific framework until checked.

- Do not claim DTAC, DSPT, NICE, NHS approval, clinical validation or procurement readiness until evidenced.
- Do not claim cost savings or clinical outcomes before the demonstrator produces evidence.
- Use HCI/P4H numbers as source notes and verify before external circulation.
- The UK legislation is the Procurement Act 2023, in force from February 2025; the HCI deck refers to the procurement reforms in its own slide language.

Useful source prompts to keep alive:

Source idea	OTOS implication
Buyers ignore generic outreach	Make every email buyer-specific and route-specific.
PINs are published early	Create PIN monitoring / framework watch task.
Social value is scored	Build a measurable social value appendix.
Frameworks carry large value share	Do not rely only on local partner warmth; map procurement route.
Regional hubs suit complex propositions	Position OTOS as implementation-heavy continuity infrastructure.
P4H enables direct buyer contact	Use the summit as a route-intelligence opportunity, not a sales stand.

13. Immediate next moves

Timing	Move
Today	Add a 1-page procurement route appendix to Executive Overview and Public Health Cost Case.
This week	Build a target route list: NHS LPP, Health Innovation East, ICB digital/prevention leads, local authority commissioners, relevant frameworks/PINs.
This week	Create a social value appendix with SMART commitments.
Before outreach	Prepare a 150-word route-advice email and a 1-page no-risk demonstrator summary.
Before any claim	Verify all HCI numbers, Procurement Act references, framework routes and eligibility.

Bottom line

This could be a direct route, but not as a blunt pitch. The strongest move is a procurement-aware “route advice” pack: OTOS proves it understands NHS buyer priorities, social value, SME access, early engagement and low-risk implementation before asking anyone to buy anything.

Appendix A - Copy block: route-advice email

Subject options:

- Route advice request - low-disruption continuity demonstrator aligned to prevention / lower acuity
- Early market engagement question - non-clinical continuity layer for ADHD-driven addiction pathway
- Who should review a bounded continuity demonstrator before any procurement discussion?

Email block:

Route-advice email Hello [Name], I am writing to ask for route advice rather than procurement. OTOS Continuity™ is a non-clinical continuity layer designed to sit around existing addiction, mental-health, ADHD, VCSE and community services. It does not diagnose, prescribe, treat, replace clinical decision-making or write into NHS records. Its first value is process visibility: handoff receipts, drift signals, consent-led engagement prompts, human review and a claims-safe learning pack. The reason I am contacting you is that the proposal appears to align with NHS priorities around prevention, lower acuity, reduced administrative drag, pathway standardisation and measurable social value. We are preparing a bounded 50-person / 12-week demonstrator and want to understand the correct route before approaching the wrong buyer or forcing this into the wrong procurement channel. Could you advise whether this belongs in early market engagement, a relevant framework/PIN route, a regional hub conversation, an innovation route, or a local prevention / population-health discussion? The first ask is simply a 20-30 minute routing conversation, not adoption, procurement or endorsement. Kind regards, Dean Butler Use note: Replace bracketed name and route details before use.
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Appendix B - Terms to add across future PDFs

Term / phrase	Why it helps
Early market engagement	Signals you understand buyer process before tender.
Route advice	Less threatening than "sales meeting."
Low-disruption demonstrator	Reduces perceived implementation risk.
Process feasibility	Avoids premature clinical outcome claims.
Continuity visibility	Turns OTOS from vague support into a mechanism.
Handoff receipts	Procurement-friendly measurable output.
Drift visibility	Useful, memorable, mechanism-led.
Human review loop	Safety control.
No merged EPR / no clinical note sharing	Buyer governance reassurance.
Specific, measurable, accountable, transparent social value	Aligns to scored procurement language.